# FAX TRANSMISSION COVER SHEET

Ridgeline Orthopedic & Spine Associates
4420 Meridian Parkway, Suite 210 | Fax: (614) 555-0198 | Phone: (614) 555-0142

| | |
|---|---|
| **TO:** | Concord Utilization Management |
| **DATE:** | 09/24/2026 |
| **FAX:** | 1-800-555-0177 |
| **PAGES:** | 106 (including cover) |
| **FROM:** | Dr. Amanda Reyes, MD |
| **RE:** | Prior Authorization Request |
| **PATIENT:** | Doe, John |
| **MEMBER ID:** | MBR-0042 |

**URGENT — Prior authorization requested for scheduled inpatient admission.**

Attached please find the complete clinical record supporting the above-referenced request, including primary care records, surgical consultation, imaging, physical therapy and pain management records, history and physical, cardiology clearance, anesthesia evaluation, laboratory results, and pre-operative orders. Please contact our office with any questions regarding this submission.

## Contents


| Document | Pages |
|---|---|
| Prior authorization request & eligibility | 2-5 |
| Primary care records & referral | 6-15 |
| Surgical consultation & spine follow-up | 16-23 |
| Imaging & electrodiagnostic studies | 24-30 |
| Physical therapy records | 31-61 |
| Pain management & injection records | 62-68 |
| History & physical | 69-71 |
| Cardiology records & clearance | 72-80 |
| Endocrinology | 81-82 |
| Anesthesia pre-operative evaluation | 83-86 |
| Laboratory results | 87-92 |
| Medication reconciliation | 93 |
| Pre-operative orders & post-operative plan | 94-97 |
| Consents, education & nursing assessment | 98-105 |
| Provider attestation | 106 |


CONFIDENTIALITY NOTICE: This facsimile transmission contains confidential health information protected under HIPAA. If you are not the intended recipient, please notify the sender immediately and destroy this transmission.

# Prior Authorization Request Form



| | |
|---|---|
| **Member Name:** | John Doe |
| **Member ID:** | MBR-0042 |
| **Date of Birth:** | 03/14/1955 |
| **Plan:** | Concord Gold Plus HMO |
| **Requesting Provider:** | Amanda Reyes, MD |
| **NPI:** | 1780234561 |
| **Facility:** | Ridgeline Surgical Center |
| **Facility NPI:** | 1922004471 |
| **Requested Service:** | Elective inpatient admission |
| **Procedure:** | Open lumbar spinal fusion, L4-L5 |
| **CPT:** | 22612 |
| **Requested Admission Date:** | 10/02/2026 |
| **Requested LOS:** | See attached notes |
| **Diagnosis:** | Lumbar spondylolisthesis with radiculopathy |
| **ICD-10:** | M43.16 |

### Clinical Justification Summary

Patient is a 71-year-old male with progressive lumbar radiculopathy refractory to conservative management (physical therapy x 12 weeks, epidural steroid injection x2). MRI demonstrates severe L4-L5 spondylolisthesis with foraminal stenosis. Surgical intervention recommended given failure of conservative treatment and progressive neurologic symptoms. See attached surgical consultation, H&P, and cardiology clearance for full clinical detail.

# Prior Authorization Request Form (continued)



### Requesting Provider Contact

| | |
|---|---|
| **Contact Name:** | Linda Morales, Surgical Scheduler |
| **Phone:** | (614) 555-0142 ext. 214 |
| **Fax:** | (614) 555-0198 |
| **Preferred Response:** | Provider portal and fax |

### Request Type

| | |
|---|---|
| **Review Type:** | Expedited — admission date within 10 days |
| **Setting:** | Inpatient hospital |
| **Place of Service:** | 21 — Inpatient hospital |
| **Additional CPT Codes:** | 22840 (posterior instrumentation), 63047 (laminectomy/decompression) |
| **Secondary Diagnoses:** | I50.22 (chronic systolic heart failure), E11.9 (type 2 diabetes), I10 (hypertension) |

### Supporting Documentation Enclosed

- Primary care records and referral
- Surgical consultation and spine follow-up notes
- Imaging reports (radiographs, MRI, flexion-extension films) and EMG/NCS
- Physical therapy evaluation, visit notes, and discharge summary
- Pain management consultation and epidural steroid injection records
- History & physical, cardiology clearance, anesthesia evaluation
- Laboratory results, medication reconciliation, pre-operative orders
- Signed consents and provider attestation

*Submitted by: Linda Morales on behalf of Amanda Reyes, MD | 09/24/2026 10:12 AM*

# Member Eligibility & Benefits Verification



| | |
|---|---|
| **Member Name:** | John Doe |
| **Member ID:** | MBR-0042 |
| **Group Number:** | GRP-88214 |
| **Effective Date:** | 01/01/2026 |
| **Plan Type:** | Concord Gold Plus HMO (Medicare Advantage) |
| **PCP of Record:** | Michael Tran, MD — Lakeview Family Medicine |
| **Prior Auth Required:** | Yes — inpatient surgical admission |
| **Inpatient Benefit:** | $350 copay per admission |
| **Annual Deductible:** | $500 (in-network); $180 met YTD |
| **Out-of-Pocket Max:** | $4,900 (in-network); $1,265 met YTD |
| **Verification Date:** | 09/20/2026 |
| **Verified By:** | K. Alvarez, Patient Access Rep |

Note: Eligibility and benefit information is verified as of the date shown above and is subject to change. Verification does not guarantee payment.

# Copy of Member Insurance Card



| | |
|---|---|
| **Plan:** | Concord Gold Plus HMO (Medicare Advantage) |
| **Member:** | JOHN DOE |
| **Member ID:** | MBR-0042 |
| **Group:** | GRP-88214 |
| **PCP:** | Michael Tran, MD (614) 555-0110 |
| **Copays:** | PCP $0 \| Specialist $35 \| ER $90 \| Inpatient $350/admission |
| **Rx BIN / PCN:** | 610014 / CONMA |
| **Member Services:** | 1-800-555-0150 (TTY 711) |
| **Prior Authorization:** | Fax 1-800-555-0177 \| Provider portal |

Card front and back copied by Ridgeline Surgical Center Patient Access, 09/20/2026.

## Referral Letter — Spine Surgery



Date: 03/12/2026

To: Amanda Reyes, MD, Ridgeline Orthopedic & Spine Associates

Re: John Doe, DOB 03/14/1955

Dear Dr. Reyes,

I am referring Mr. Doe, a 71-year-old man with progressive low back pain radiating into the right leg, for surgical evaluation. Radiographs show a grade 1 anterolisthesis at L4-L5. He has right L5 radicular findings on examination and has begun a course of physical therapy.

His history is notable for heart failure with reduced ejection fraction followed by Heartland Cardiology, type 2 diabetes, hypertension, and hyperlipidemia. He is a former smoker. He is motivated and lives with his spouse, who is supportive.

Enclosed are my recent office notes and imaging. Thank you for seeing him.

*Michael Tran, MD | Lakeview Family Medicine | (614) 555-0110*

# Problem List & Health Maintenance



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Printed: 08/25/2026

### Active Problems

| Problem | ICD-10 | Onset |
|---|---|---|
| Chronic systolic heart failure | I50.22 | 2022 |
| Type 2 diabetes mellitus | E11.9 | 2014 |
| Essential hypertension | I10 | 2010 |
| Hyperlipidemia | E78.5 | 2010 |
| Gastroesophageal reflux disease | K21.9 | 2016 |
| Lumbar radiculopathy | M54.16 | 2026 |
| Spondylolisthesis, lumbar region | M43.16 | 2026 |

### Immunizations

Influenza 10/2025. Pneumococcal (PCV20) 2023. COVID-19 booster 09/2025. Tdap 2019. Shingles series complete 2021.

### Screening

Colonoscopy 2021, normal, repeat 2031. Diabetic eye exam 03/2026, no retinopathy. Diabetic foot exam 03/2026, diminished monofilament sensation right great toe.

# Annual Wellness Visit



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 01/15/2026

**Summary**

Annual wellness visit. Reports intermittent low back stiffness for several weeks, no leg symptoms at that time. Independent with all activities of daily living. Walks 20 minutes daily.

**Functional and Safety Screen**

No falls in the past year. Lives with spouse in a two-story home. Hearing and vision adequate with glasses. Depression screen (PHQ-2) negative.

**Plan**

1. Continue current medications.
2. Back stiffness: stretching program; return if worse or if leg symptoms develop.
3. Annual labs due in June.

*Electronically signed: Michael Tran, MD | 01/15/2026*

# Office Visit Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 02/10/2026

**Reason for Visit**

Low back pain, 4 weeks

**History of Present Illness**

Gradual-onset low back pain without trauma, radiating intermittently into the right buttock. No bowel or bladder changes. Taking acetaminophen with partial relief.

**Examination**

BP 136/80, HR 74, Wt 212 lb. Lumbar paraspinal tenderness. Straight leg raise equivocal on right. Strength 5/5.

**Assessment**

Mechanical low back pain with possible right L5 radicular component.

**Plan**

1. Lumbar spine radiographs ordered.
2. Acetaminophen 1 g TID as needed; avoid NSAIDs given heart failure and renal risk.
3. Home exercise handout provided. Return in 4 weeks.

*Electronically signed: Michael Tran, MD | 02/10/2026*

# Office Visit Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 03/12/2026

**Reason for Visit**

Follow-up, back pain worse

**History of Present Illness**

Pain now radiating below the right knee to the dorsum of the foot. Reports tingling in the right great toe and difficulty walking more than two blocks.

**Examination**

BP 140/84, HR 78. Positive straight leg raise on right at 45 degrees. EHL strength 4+/5 right.

**Assessment**

Lumbar radiculopathy, right L5 distribution, with grade 1 spondylolisthesis on radiographs.

**Plan**

1. Referral to physical therapy, 2x/week for 12 weeks.
2. Referral to spine surgery for evaluation.
3. Gabapentin 100 mg at bedtime, titrate as tolerated.

*Electronically signed: Michael Tran, MD | 03/12/2026*

# Office Visit Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 04/14/2026

**Reason for Visit**

Chronic disease follow-up

**History of Present Illness**

Heart failure symptoms stable, mild exertional dyspnea unchanged. Back pain persists; started physical therapy last week.

**Examination**

BP 134/78, HR 72, Wt 210 lb. Lungs clear. Trace ankle edema. Lumbar exam unchanged.

**Assessment**

HFrEF, stable. Type 2 diabetes, A1c pending. Lumbar radiculopathy, in PT.

**Plan**

1. Continue guideline-directed medical therapy per cardiology.
2. Gabapentin increased to 300 mg at bedtime.
3. Labs at next visit.

*Electronically signed: Michael Tran, MD | 04/14/2026*

# Office Visit Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 06/02/2026

**Reason for Visit**

Follow-up, labs reviewed

**History of Present Illness**

Six weeks of PT completed with minimal change. Pain 6/10, worse with standing. Sleep disrupted by leg pain.

**Examination**

BP 138/82, HR 76. Diminished right Achilles reflex. Sensation reduced over right L5 dermatome.

**Assessment**

Persistent right L5 radiculopathy despite conservative care. A1c 7.3%.

**Plan**

1. Referral to pain management for epidural steroid injection.
2. Continue PT to complete 12-week course.
3. Diabetes: continue metformin and glipizide; dietary counseling.

*Electronically signed: Michael Tran, MD | 06/02/2026*

# Office Visit Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 07/20/2026

**Reason for Visit**

Follow-up after first injection

**History of Present Illness**

First epidural steroid injection on 06/10/2026 gave about two weeks of partial relief; pain has returned to baseline. Second injection scheduled 07/22/2026.

**Examination**

BP 132/80, HR 70. Neurologic findings unchanged from prior visit.

**Assessment**

Lumbar radiculopathy, refractory.

**Plan**

1. Proceed with second injection as planned.
2. MRI lumbar spine to be ordered by spine surgeon.

*Electronically signed: Michael Tran, MD | 07/20/2026*

# Office Visit Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 08/25/2026

**Reason for Visit**

Pre-surgical planning

**History of Present Illness**

Second injection provided minimal and short-lived relief. Patient now using a cane and reports difficulty with stairs at home. Wishes to proceed with surgical evaluation.

**Examination**

BP 138/82, HR 76, Wt 208 lb. Antalgic gait with single-point cane.

**Assessment**

Lumbar spondylolisthesis with radiculopathy, failed conservative management.

**Plan**

1. Agree with surgical consultation.
2. Cardiology pre-operative clearance requested given HFrEF.
3. Records forwarded to Ridgeline Orthopedic & Spine.

*Electronically signed: Michael Tran, MD | 08/25/2026*

# Office Visit Note — Pre-Operative Medical Evaluation



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 09/15/2026

**Reason for Visit**

Medical optimization before planned lumbar fusion on 10/02/2026.

**Assessment**

Medically optimized from a primary care standpoint pending cardiology and anesthesia evaluation. Blood pressure at goal. Diabetes control acceptable for elective surgery.

**Plan**

1. Cardiology appointment 09/16/2026 for clearance.
2. Pre-admission testing at Ridgeline on 09/22/2026.
3. Arranged for spouse and daughter to assist at home after discharge.

*Electronically signed: Michael Tran, MD | 09/15/2026*

# Surgical Consultation Note



Patient: John Doe | DOB: 03/14/1955 | Date of Visit: 09/18/2026

### Chief Complaint

Progressive low back pain with right leg radiculopathy, 8 months duration.

### History of Present Illness

Mr. Doe is a 71-year-old male referred for evaluation of chronic low back pain with radiation into the right lower extremity. Symptoms have progressively worsened over the past 8 months despite a course of physical therapy and two epidural steroid injections. MRI lumbar spine dated 08/15/2026 demonstrates Grade 1 spondylolisthesis at L4-L5 with associated foraminal stenosis and nerve root impingement. Patient reports increasing difficulty with ambulation and activities of daily living.

### Past Medical History

Congestive heart failure (reduced ejection fraction, last EF 40% per cardiology), Type 2 diabetes mellitus, hypertension, hyperlipidemia.

### Assessment

Lumbar spondylolisthesis with radiculopathy, refractory to conservative management. Surgical candidate for decompression and fusion.

### Plan

Recommend open lumbar spinal fusion, L4-L5, with posterior instrumentation. Given patient's cardiac history (reduced EF) and diabetes, cardiology clearance obtained prior to scheduling — see attached. Surgeon expects a multi-day stay for pain control and mobilization. Patient counseled on risks, benefits, and alternatives; agrees to proceed.

*Electronically signed: Amanda Reyes, MD | 09/18/2026 4:42 PM*

# Surgical Consultation Note (continued)



### Review of Systems

Constitutional: Denies fever, chills, unintentional weight loss. Cardiovascular: Denies chest pain, orthopnea. Reports mild exertional dyspnea, stable. Endocrine: Diabetes controlled on oral agents, last A1c 7.1%. Musculoskeletal: As above.

### Physical Examination

Vitals: BP 138/82, HR 76, RR 16, SpO2 97% RA. Lumbar spine with decreased range of motion, positive straight leg raise on right. Motor strength 4/5 right EHL. Sensation diminished right L5 dermatome. Reflexes 1+ bilateral patellar, absent right Achilles.

### Imaging Reviewed

MRI Lumbar Spine (08/15/2026): Grade 1 anterolisthesis L4 on L5, severe bilateral foraminal stenosis, moderate central canal stenosis. Flexion-extension radiographs (09/02/2026): 4 mm of translation at L4-L5, consistent with dynamic instability.

### Conservative Treatment History

| Treatment | Dates | Response |
|---|---|---|
| Physical therapy, 24 visits | 05/04/2026 – 07/24/2026 | Minimal functional improvement |
| Epidural steroid injection #1 | 06/10/2026 | ~40% relief for 2 weeks |
| Epidural steroid injection #2 | 07/22/2026 | <20% relief for ~10 days |
| Gabapentin, acetaminophen | 03/2026 – present | Partial, limited by sedation |

# Surgical Consultation Note (continued)



### Surgical Discussion

Given dynamic instability at L4-L5 with severe foraminal stenosis and a progressive motor deficit, decompression alone carries a meaningful risk of worsening instability. Instrumented posterior fusion with decompression is recommended.

### Risks Discussed

Infection, bleeding, dural tear, nerve injury, nonunion, adjacent segment disease, blood clots, and cardiac and anesthetic risk related to heart failure. Patient verbalized understanding.

### Alternatives Discussed

Continued non-operative care, additional injections, and decompression without fusion. Patient declines further non-operative management.

### Pre-Operative Requirements

1. Cardiology pre-operative clearance.
2. Anesthesia pre-admission evaluation.
3. Endocrinology review of peri-operative diabetes management.
4. Pre-admission laboratory testing.

*Electronically signed: Amanda Reyes, MD | 09/18/2026 4:42 PM*

## Spine Clinic — New Patient Visit



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 04/02/2026

**History**

Referred by Dr. Tran for right leg pain and grade 1 spondylolisthesis on radiographs. Began physical therapy this month.

**Examination**

Positive straight leg raise on right. EHL 4+/5 right. Reflexes symmetric at this visit.

**Assessment / Plan**

Right L5 radiculopathy with degenerative spondylolisthesis. Recommend completing a full 12-week course of physical therapy and considering epidural steroid injection. Return after PT, or sooner if weakness progresses.

*Electronically signed: Amanda Reyes, MD | 04/02/2026*

## Spine Clinic — Follow-up Visit



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 06/18/2026

**Interval History**

Six weeks into physical therapy. First injection last week with partial relief so far.

**Examination**

EHL 4+/5 right, unchanged. Right Achilles reflex diminished.

**Plan**

Complete physical therapy and reassess after the injection series.

*Electronically signed: Amanda Reyes, MD | 06/18/2026*

## Spine Clinic — Follow-up Visit



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Visit: 08/06/2026

**Interval History**

Completed 12 weeks of PT and two epidural steroid injections without durable relief. Now walking with a cane.

**Examination**

EHL strength decreased to 4/5 right. Right Achilles reflex now absent. Sensory loss over right L5 dermatome.

**Assessment / Plan**

Progressive motor deficit despite conservative care. MRI lumbar spine and flexion-extension radiographs ordered. EMG/NCS of the right lower extremity ordered. Surgical consultation to follow.

*Electronically signed: Amanda Reyes, MD | 08/06/2026*

# Patient-Reported Outcome Measures



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Collected: 09/18/2026

| Measure | 04/02/2026 | 09/18/2026 | Interpretation |
|---|---|---|---|
| Oswestry Disability Index | 46% | 56% | Severe disability |
| Back pain (NPRS) | 5/10 | 6/10 | Worsened |
| Leg pain (NPRS) | 6/10 | 8/10 | Worsened |
| PROMIS physical function | 38 | 32 | Below population mean |

Scores worsened despite twelve weeks of physical therapy and two injections.

# Surgical Scheduling Request



| | |
|---|---|
| **Patient:** | John Doe, DOB 03/14/1955 |
| **Surgeon:** | Amanda Reyes, MD |
| **Procedure:** | Open lumbar spinal fusion, L4-L5, posterior instrumentation (CPT 22612, 22840, 63047) |
| **Facility:** | Ridgeline Surgical Center |
| **Requested Date:** | 10/02/2026, first case |
| **Estimated Duration:** | 4 hours |
| **Admission:** | Inpatient, telemetry bed |
| **Equipment:** | Pedicle screw system, neuromonitoring, cell saver |
| **Prior Auth Status:** | Pending — submitted 09/24/2026 |

*Linda Morales, Surgical Scheduler | 09/19/2026*

# Radiology Report — Lumbar Spine Radiographs



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Exam Date: 02/20/2026 | Ordering: Michael Tran, MD

**Exam**

XR lumbar spine, AP and lateral, 2 views.

**Findings**

Grade 1 anterolisthesis of L4 on L5. Moderate disc space narrowing at L4-L5 and L5-S1 with endplate sclerosis. Facet arthropathy at L4-L5. No acute fracture. Vascular calcification of the abdominal aorta.

**Impression**

Degenerative grade 1 spondylolisthesis at L4-L5 with multilevel degenerative disc disease.

*Electronically signed: Grace Whitfield, MD (Radiology) | 02/20/2026*

# Radiology Report — MRI Lumbar Spine without Contrast



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Exam Date: 08/15/2026 | Ordering: Amanda Reyes, MD

**Clinical History**

Right L5 radiculopathy, failed conservative management.

**Technique**

Multiplanar, multisequence MRI of the lumbar spine without intravenous contrast.

**Findings**

L3-L4: Mild disc bulge. No significant stenosis.

L4-L5: Grade 1 anterolisthesis. Disc uncovering with broad-based bulge. Severe facet hypertrophy with ligamentum flavum thickening. Moderate central canal stenosis. Severe bilateral foraminal stenosis with impingement of the exiting right L4 and traversing right L5 nerve roots.

L5-S1: Mild disc desiccation. Mild left foraminal narrowing.

Conus terminates normally at L1. No marrow signal abnormality.

# Radiology Report — MRI Lumbar Spine (continued)



**Impression**

1. Grade 1 anterolisthesis of L4 on L5 with severe bilateral foraminal stenosis.
2. Moderate central canal stenosis at L4-L5.
3. Findings correlate with the clinical right L5 radiculopathy.

*Electronically signed: Grace Whitfield, MD (Radiology) | 08/15/2026*

# Radiology Report — Lumbar Flexion-Extension Radiographs



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Exam Date: 09/02/2026 | Ordering: Amanda Reyes, MD

**Findings**

Neutral: Grade 1 anterolisthesis of L4 on L5 measuring 6 mm. Flexion: slip increases to 10 mm. Extension: slip reduces to 6 mm. Total translation of 4 mm at L4-L5.

**Impression**

Dynamic instability at L4-L5.

*Electronically signed: Grace Whitfield, MD (Radiology) | 09/02/2026*

# EMG / Nerve Conduction Study Report



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Study Date: 08/27/2026 | Referring: Amanda Reyes, MD

### Nerve Conduction Studies

| Nerve | Latency | Amplitude | Velocity |
|---|---|---|---|
| R peroneal motor | 5.8 ms | 1.9 mV (low) | 41 m/s |
| R tibial motor | 5.1 ms | 6.2 mV | 43 m/s |
| R superficial peroneal sensory | 3.9 ms | 7 uV | 44 m/s |
| R sural sensory | 4.0 ms | 8 uV | 42 m/s |

### Needle EMG

Fibrillation potentials and positive sharp waves in the right tibialis anterior, extensor hallucis longus, and L5 paraspinals. Chronic reinnervation changes in the same muscles.

### Interpretation

Findings consistent with chronic right L5 radiculopathy with ongoing denervation. No evidence of peripheral neuropathy.

*Electronically signed: Samuel Okafor, MD (Neurology) | 08/27/2026*

# Radiology Report — Chest Radiograph (Pre-Operative)



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Exam Date: 09/22/2026

**Findings**

Mild cardiomegaly, unchanged. No pulmonary edema. No focal consolidation or effusion. Degenerative changes of the thoracic spine.

**Impression**

Stable mild cardiomegaly. No acute cardiopulmonary process.

*Electronically signed: Grace Whitfield, MD (Radiology) | 09/22/2026*

# Fluoroscopy Report — Spinal Injection Guidance



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Dates: 06/10/2026 and 07/22/2026

**Findings**

Fluoroscopic guidance for right L4-L5 transforaminal epidural injections. Contrast outlines the right L5 nerve root with epidural spread on both dates. Fluoroscopy time 38 seconds and 41 seconds.

*Electronically signed: Grace Whitfield, MD (Radiology) | 07/22/2026*

# Physical Therapy Initial Evaluation



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 05/04/2026 | Referring: Michael Tran, MD

**Subjective**

Low back pain with right leg radiation for 3 months. Pain 7/10 at worst. Walking tolerance two blocks. Difficulty with prolonged standing, stairs, and yard work.

**Objective**

Lumbar flexion limited to 50% with reproduction of leg pain. Positive right straight leg raise. Hip abductor strength 4/5 bilaterally. Oswestry Disability Index (ODI): 52%.

**Assessment**

Signs and symptoms consistent with lumbar radiculopathy with movement impairment. Good rehab potential for symptom management.

**Plan**

Twice weekly for 12 weeks: neural mobilization, core stabilization, gait training, and home exercise program.

*Electronically signed: Rachel Kim, PT, DPT | 05/04/2026*

# Physical Therapy Initial Evaluation (continued)



### Goals (12 weeks)

1. Reduce worst pain to 3/10 or less.
2. Improve walking tolerance to 30 minutes.
3. Reduce ODI to below 30%.
4. Independent with home exercise program.

### Precautions

Heart failure: monitor for dyspnea and edema; stop exercise if SpO2 drops below 90% or symptoms worsen. Diabetes: confirm glucose above 100 mg/dL before session.

### Plan of Care Certification

Plan of care reviewed and certified by referring physician.

*Certified: Michael Tran, MD | 05/06/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 1 of 24 | Date: 05/05/2026

**Subjective**

Worst pain 7/10 over the past several days. Walking tolerance about 8 minutes before leg pain forces rest.

**Treatment**

Neural mobilization (sciatic slider), supine core activation, gait assessment.

**Response**

Leg pain eased during session; returned within an hour.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 05/05/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 2 of 24 | Date: 05/08/2026

**Subjective**

Worst pain 7/10 over the past several days. Walking tolerance about 8 minutes before leg pain forces rest.

**Treatment**

Core stabilization, hip abductor strengthening, home exercise review.

**Response**

Tolerated session; reports soreness the following day.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 05/08/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 3 of 24 | Date: 05/12/2026

**Subjective**

Worst pain 7/10 over the past several days. Walking tolerance about 10 minutes before leg pain forces rest.

**Treatment**

Aquatic-style walking program progression, lumbar flexion bias exercises.

**Response**

Unable to progress walking distance due to right leg pain.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 05/12/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 4 of 24 | Date: 05/15/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 10 minutes before leg pain forces rest.

**Treatment**

Stationary cycling 10 minutes, bridges, neural mobilization.

**Response**

Mild improvement in morning stiffness; leg pain unchanged.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 05/15/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 5 of 24 | Date: 05/19/2026

**Subjective**

Worst pain 7/10 over the past several days. Walking tolerance about 10 minutes before leg pain forces rest.

**Treatment**

Balance training, sit-to-stand practice, postural education.

**Response**

Symptoms unchanged. Patient frustrated with lack of progress.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 05/19/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 6 of 24 | Date: 05/22/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Core progression, step-ups 4-inch step, walking program.

**Response**

Increased numbness in right foot after prolonged standing.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 05/22/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 7 of 24 | Date: 05/26/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Neural mobilization (sciatic slider), supine core activation, gait assessment.

**Response**

Leg pain eased during session; returned within an hour.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 05/26/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 8 of 24 | Date: 05/29/2026

**Subjective**

Worst pain 7/10 over the past several days. Walking tolerance about 10 minutes before leg pain forces rest.

**Treatment**

Core stabilization, hip abductor strengthening, home exercise review.

**Response**

Tolerated session; reports soreness the following day.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 05/29/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 9 of 24 | Date: 06/02/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Aquatic-style walking program progression, lumbar flexion bias exercises.

**Response**

Unable to progress walking distance due to right leg pain.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 06/02/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 10 of 24 | Date: 06/05/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Stationary cycling 10 minutes, bridges, neural mobilization.

**Response**

Mild improvement in morning stiffness; leg pain unchanged.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 06/05/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 11 of 24 | Date: 06/09/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Balance training, sit-to-stand practice, postural education.

**Response**

Symptoms unchanged. Patient frustrated with lack of progress.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 06/09/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 12 of 24 | Date: 06/12/2026

**Subjective**

Worst pain 5/10 over the past several days. Walking tolerance about 15 minutes before leg pain forces rest.

**Treatment**

Core progression, step-ups 4-inch step, walking program.

**Response**

Increased numbness in right foot after prolonged standing.

Note: Patient received epidural steroid injection on 06/10/2026; reports improved leg pain this week.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 06/12/2026*

*[ This page intentionally left blank — fax transmission ]*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 13 of 24 | Date: 06/16/2026

**Subjective**

Worst pain 5/10 over the past several days. Walking tolerance about 15 minutes before leg pain forces rest.

**Treatment**

Neural mobilization (sciatic slider), supine core activation, gait assessment.

**Response**

Leg pain eased during session; returned within an hour.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 06/16/2026*

## Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 14 of 24 | Date: 06/19/2026

**Subjective**

Worst pain 5/10 over the past several days. Walking tolerance about 15 minutes before leg pain forces rest.

**Treatment**

Core stabilization, hip abductor strengthening, home exercise review.

**Response**

Tolerated session; reports soreness the following day.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 06/19/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 15 of 24 | Date: 06/23/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Aquatic-style walking program progression, lumbar flexion bias exercises.

**Response**

Unable to progress walking distance due to right leg pain.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 06/23/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 16 of 24 | Date: 06/26/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Stationary cycling 10 minutes, bridges, neural mobilization.

**Response**

Mild improvement in morning stiffness; leg pain unchanged.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 06/26/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 17 of 24 | Date: 06/30/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Balance training, sit-to-stand practice, postural education.

**Response**

Symptoms unchanged. Patient frustrated with lack of progress.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 06/30/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 18 of 24 | Date: 07/03/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Core progression, step-ups 4-inch step, walking program.

**Response**

Increased numbness in right foot after prolonged standing.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 07/03/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 19 of 24 | Date: 07/07/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Neural mobilization (sciatic slider), supine core activation, gait assessment.

**Response**

Leg pain eased during session; returned within an hour.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 07/07/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 20 of 24 | Date: 07/10/2026

**Subjective**

Worst pain 7/10 over the past several days. Walking tolerance about 10 minutes before leg pain forces rest.

**Treatment**

Core stabilization, hip abductor strengthening, home exercise review.

**Response**

Tolerated session; reports soreness the following day.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 07/10/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 21 of 24 | Date: 07/14/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Aquatic-style walking program progression, lumbar flexion bias exercises.

**Response**

Unable to progress walking distance due to right leg pain.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 07/14/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 22 of 24 | Date: 07/17/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Stationary cycling 10 minutes, bridges, neural mobilization.

**Response**

Mild improvement in morning stiffness; leg pain unchanged.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 07/17/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 23 of 24 | Date: 07/21/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Balance training, sit-to-stand practice, postural education.

**Response**

Symptoms unchanged. Patient frustrated with lack of progress.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 07/21/2026*

# Physical Therapy Daily Note



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Visit 24 of 24 | Date: 07/24/2026

**Subjective**

Worst pain 6/10 over the past several days. Walking tolerance about 12 minutes before leg pain forces rest.

**Treatment**

Core progression, step-ups 4-inch step, walking program.

**Response**

Increased numbness in right foot after prolonged standing.

Final visit of plan of care. See discharge summary.

**Plan**

Continue plan of care. Next visit as scheduled.

*Rachel Kim, PT, DPT | 07/24/2026*

# Physical Therapy Progress Note — Week 6



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 06/12/2026

### Progress Toward Goals

| Goal | Baseline | Current | Status |
|---|---|---|---|
| Worst pain | 7/10 | 5/10 | Partially met |
| Walking tolerance | 8 min | 15 min | Not met |
| ODI | 52% | 48% | Not met |
| Home program | Not started | Independent | Met |

### Assessment

Limited improvement. Temporary benefit after the epidural injection this week. Recommend continuing plan of care.

*Rachel Kim, PT, DPT | 06/12/2026*

# Physical Therapy Discharge Summary



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 07/24/2026

### Summary

Patient completed 24 visits over 12 weeks with minimal functional improvement. Gains from the first injection were not sustained.

### Outcome Measures

| Measure | Initial (05/04) | Discharge (07/24) |
|---|---|---|
| Worst pain (NPRS) | 7/10 | 6/10 |
| Walking tolerance | 8 minutes | 12 minutes |
| Oswestry Disability Index | 52% | 50% |

### Recommendation

Plateau in conservative management. Recommend follow-up with spine surgery. Continue home exercise program.

*Rachel Kim, PT, DPT | 07/24/2026*

## Home Exercise Program



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Issued: 05/04/2026, updated 06/12/2026

| Exercise | Sets / Reps | Frequency |
|---|---|---|
| Sciatic nerve slider, supine | 2 x 10 | Twice daily |
| Abdominal bracing | 10 x 10-second holds | Twice daily |
| Bridges | 2 x 10 | Daily |
| Side-lying hip abduction | 2 x 10 each side | Daily |
| Walking program | Start 8 minutes, add 2 minutes weekly | Daily |

Stop any exercise that increases leg numbness or weakness, and contact your therapist.

# Physical Therapy Attendance Log



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Plan of care: 05/04/2026 – 07/24/2026

| Scheduled | Attended | Cancelled | No-show |
|---|---|---|---|
| 24 | 24 | 0 | 0 |

Attendance 100%. Patient adherent to home exercise program per self-report and therapist observation.

*Front desk verification: Summit Rehab & Physical Therapy | 07/24/2026*

# Pain Management Consultation



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 06/03/2026 | Referring: Michael Tran, MD

**History**

Right L5 radicular pain for 4 months. Six weeks of PT with minimal benefit. Gabapentin limited by daytime sedation. NSAIDs avoided due to heart failure.

**Examination**

Positive right straight leg raise. Diminished sensation over the right dorsal foot. EHL 4+/5 right.

**Assessment**

Lumbar radiculopathy secondary to L4-L5 foraminal stenosis.

**Plan**

Right L4-L5 transforaminal epidural steroid injection. Glucose monitoring after injection given diabetes. Up to two injections if partial response.

*Electronically signed: Kevin Osei, MD | 06/03/2026*

## Consent for Epidural Steroid Injection



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 06/10/2026

I consent to a right L4-L5 transforaminal epidural steroid injection under fluoroscopy by Kevin Osei, MD. Risks explained include infection, bleeding, headache, temporary numbness or weakness, elevated blood sugar, nerve injury, and lack of benefit.

*Patient signature: John Doe | Witness: T. Ruiz, RN | 06/10/2026*

# Procedure Note — Epidural Steroid Injection #1



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Procedure: 06/10/2026 | Physician: Kevin Osei, MD

**Procedure**

Right L4-L5 transforaminal epidural steroid injection under fluoroscopic guidance.

**Indication**

Right L5 radiculopathy refractory to physical therapy and oral medications.

**Description**

Informed consent obtained. Patient positioned prone. Needle advanced to the right L4-L5 foramen. Contrast confirmed epidural spread without vascular uptake. Injected triamcinolone 40 mg with 1 mL 0.25% bupivacaine. No complications. Patient tolerated the procedure well.

**Peri-Procedure Notes**

Aspirin continued. Fingerstick glucose 148 mg/dL pre-procedure; patient advised steroid may raise glucose for 2-3 days.

**Pre / Post Pain Score**

Pre: 7/10. Post (30 min): 3/10.

*Electronically signed: Kevin Osei, MD | 06/10/2026*

# Post-Procedure Follow-up



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 06/24/2026

**Response**

Approximately 40% relief of leg pain lasting about two weeks, now returning. Glucose peaked at 240 mg/dL for two days post-injection, then normalized.

**Plan**

Repeat injection in 4-6 weeks if pain returns to baseline.

*Electronically signed: Kevin Osei, MD | 06/24/2026*

# Consent for Epidural Steroid Injection



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 07/22/2026

I consent to a right L4-L5 transforaminal epidural steroid injection under fluoroscopy by Kevin Osei, MD. Risks explained include infection, bleeding, headache, temporary numbness or weakness, elevated blood sugar, nerve injury, and lack of benefit.

*Patient signature: John Doe | Witness: T. Ruiz, RN | 07/22/2026*

# Procedure Note — Epidural Steroid Injection #2



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date of Procedure: 07/22/2026 | Physician: Kevin Osei, MD

**Procedure**

Right L4-L5 transforaminal epidural steroid injection under fluoroscopic guidance.

**Indication**

Right L5 radiculopathy refractory to physical therapy and oral medications.

**Description**

Informed consent obtained. Patient positioned prone. Needle advanced to the right L4-L5 foramen; repeat approach. Contrast confirmed epidural spread without vascular uptake. Injected triamcinolone 40 mg with 1 mL 0.25% bupivacaine. No complications. Patient tolerated the procedure well.

**Peri-Procedure Notes**

Aspirin continued. Fingerstick glucose 148 mg/dL pre-procedure; patient advised steroid may raise glucose for 2-3 days.

**Pre / Post Pain Score**

Pre: 7/10. Post (30 min): 3/10.

*Electronically signed: Kevin Osei, MD | 07/22/2026*

# Post-Procedure Follow-up



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 08/05/2026

**Response**

Patient reports less than 20% relief lasting approximately 10 days following the second injection.

**Assessment**

Diminishing response to injection therapy. Further injections are unlikely to provide durable benefit.

**Plan**

Return to spine surgery for operative evaluation. No further injections planned.

*Electronically signed: Kevin Osei, MD | 08/05/2026*

# History & Physical



Patient: John Doe | MRN: 774213 | Date: 09/22/2026

### Reason for Admission

Pre-operative history and physical for scheduled open lumbar spinal fusion, L4-L5.

### Past Medical History

1. Congestive heart failure, HFrEF, EF 40% (echo 07/2026)
2. Type 2 diabetes mellitus, on metformin and glipizide
3. Hypertension, controlled on lisinopril
4. Hyperlipidemia, on atorvastatin
5. GERD

### Past Surgical History

Cholecystectomy (2011), right total knee arthroplasty (2019).

### Medications

Metformin 1000mg BID, Glipizide 5mg daily, Lisinopril 20mg daily, Atorvastatin 40mg daily, Furosemide 20mg daily, Aspirin 81mg daily.

### Allergies

No known drug allergies.

### Social History

Former smoker, quit 2015 (30 pack-year history). Denies alcohol or illicit substance use. Lives with spouse, independent with ADLs at baseline.

### Family History

Father: coronary artery disease. Mother: type 2 diabetes.

### Physical Examination

General: Well-developed, well-nourished male in no acute distress.

Cardiovascular: Regular rate and rhythm, no murmurs. Trace bilateral lower extremity edema.

Pulmonary: Clear to auscultation bilaterally, no wheezes or crackles.

Abdomen: Soft, non-tender, non-distended.

Extremities: No cyanosis or clubbing. Trace edema as noted above.

Neurologic: As detailed in surgical consultation note.

### Functional Status

Patient ambulates with a single-point cane due to pain, otherwise independent. Lives in a two-story home; will require assistance with stairs post-operatively.

### Pre-Admission Testing Summary

See attached laboratory results (page 87) and cardiology clearance (pages 77-78) for complete pre-operative risk stratification.

### Anesthesia Risk Assessment

ASA Physical Status Classification: III (severe systemic disease — CHF, diabetes). Anesthesia pre-op evaluation completed 09/23/2026, cleared for surgery with standard cardiac monitoring intra-operatively.

### Assessment / Plan

71-year-old male with HFrEF (EF 40%) and type 2 diabetes, cleared for open lumbar spinal fusion following cardiology evaluation. Anticipate inpatient admission for post-operative monitoring given cardiac history; exact anticipated length of stay to be determined by surgical team based on intra-operative course and post-operative mobilization.

*Electronically signed: Daniel Brooks, MD (Hospitalist, Pre-Admission Testing) | 09/22/2026*

## Heart Failure Clinic Visit



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 01/21/2026

**Interval History**

Doing well. NYHA Class II. Home weights stable at 210-212 lb.

**Plan**

Continue lisinopril 20 mg, furosemide 20 mg, aspirin. Repeat echocardiogram in July. Return in 3 months.

*Electronically signed: Priya Nkemelu, MD, FACC | 01/21/2026*

## Heart Failure Clinic Visit



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 04/08/2026

**Interval History**

NYHA Class II. No hospitalizations since diagnosis in 2022. Weight stable. Adherent to medications and 2 g sodium diet.

**Examination**

BP 128/76, HR 68. JVP not elevated. Lungs clear. Trace ankle edema.

**Assessment / Plan**

Chronic HFrEF, compensated. Continue lisinopril, furosemide. Discussed transition to sacubitril/valsartan; patient prefers to defer given cost. Repeat echocardiogram in July.

*Electronically signed: Priya Nkemelu, MD, FACC | 04/08/2026*

# Transthoracic Echocardiogram Report



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Study Date: 07/09/2026

### Measurements

| Parameter | Value | Normal |
|---|---|---|
| LVEF (biplane) | 40% | 52-72% |
| LV end-diastolic diameter | 5.9 cm | 4.2-5.8 cm |
| Left atrial volume index | 38 mL/m2 | <34 mL/m2 |
| TAPSE | 1.9 cm | >1.7 cm |
| RVSP | 32 mmHg | <35 mmHg |

### Findings

Mildly dilated left ventricle with moderate global hypokinesis. Mild-moderate mitral regurgitation. No significant valvular stenosis. Normal right ventricular size and function. No pericardial effusion.

**Impression**

1. Moderately reduced left ventricular systolic function, LVEF 40%, unchanged from 2025.
2. Mild-moderate mitral regurgitation.
3. Normal right ventricular function.

*Electronically signed: Priya Nkemelu, MD, FACC | 07/09/2026*

## Cardiology Office Visit



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 07/09/2026

**Interval History**

Stable exertional dyspnea after two flights of stairs. Back pain limiting activity. No chest pain, syncope, or orthopnea.

**Assessment / Plan**

HFrEF, stable, LVEF 40% on today's echo. Continue current regimen. Patient may need spine surgery; will see for pre-operative risk assessment when a date is set.

*Electronically signed: Priya Nkemelu, MD, FACC | 07/09/2026*

# Cardiology Pre-Operative Clearance



Patient: John Doe | DOB: 03/14/1955 | Date: 09/16/2026

### Reason for Consultation

Pre-operative cardiac risk assessment prior to elective lumbar spinal fusion.

### History

Patient with known history of heart failure with reduced ejection fraction, last assessed at 40% on echocardiogram dated 07/09/2026. NYHA Class II symptoms, stable on current guideline-directed medical therapy. No recent decompensation, hospitalization, or change in symptoms.

### Diagnostic Studies

Echocardiogram (07/09/2026): LVEF 40%, mild-moderate mitral regurgitation, no significant valvular stenosis.

EKG (09/16/2026): Normal sinus rhythm, no acute ST-T changes.

Stress test: Not indicated given recent echo and stable symptoms.

### Assessment

Revised Cardiac Risk Index: 2 points (heart failure, diabetes). Estimated peri-operative cardiac risk: intermediate. Patient's heart failure is compensated and stable.

# Cardiology Pre-Operative Clearance (continued)



**Recommendations**

1. Patient is cleared for surgery from a cardiac standpoint.
2. Continue home cardiac medications through peri-operative period per anesthesia protocol.
3. Recommend telemetry monitoring for first 24-48 hours post-operatively given HFrEF.
4. Maintain euvolemia — monitor daily weights and strict intake/output.
5. Given cardiac history, inpatient monitoring is recommended; anticipate this will support a multi-day admission for adequate post-operative cardiac and pain management, though exact duration will depend on post-operative course.

Clearance Status: CLEARED FOR SURGERY

*Electronically signed: Dr. Priya Nkemelu, MD, FACC | 09/16/2026*

# 12-Lead Electrocardiogram



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 09/16/2026

| Measurement | Value |
|---|---|
| Rate | 74 bpm |
| Rhythm | Normal sinus rhythm |
| PR interval | 176 ms |
| QRS duration | 104 ms |
| QTc | 438 ms |
| Axis | -12 degrees |

### Interpretation

Normal sinus rhythm. Left ventricular hypertrophy by voltage criteria. No acute ST-T changes. Unchanged from 07/2026.

*Read by: Priya Nkemelu, MD, FACC | 09/16/2026*

## Cardiology Letter to Surgeon



Date: 09/16/2026

To: Amanda Reyes, MD

Re: John Doe, DOB 03/14/1955

Dear Dr. Reyes, I saw Mr. Doe today for pre-operative assessment. My full clearance note is enclosed. In brief, his heart failure is compensated, and I recommend telemetry and close fluid management after surgery. Please call me with any peri-operative concerns.

*Priya Nkemelu, MD, FACC | (614) 555-0177 ext. 3*

# Endocrinology Consultation — Peri-Operative Diabetes



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 09/19/2026

**History**

Type 2 diabetes for 12 years on metformin and glipizide. A1c 7.3% (06/2026), 7.1% (09/2026). No hypoglycemia. Glucose rose to 240 mg/dL after each epidural steroid injection.

**Assessment**

Type 2 diabetes, adequately controlled for elective surgery (A1c below 8%). Risk of peri-operative hyperglycemia with surgical stress.

**Recommendations**

1. Hold glipizide the morning of surgery.
2. Metformin may be continued per anesthesia protocol; hold if contrast is given.
3. Sliding scale insulin while NPO and post-operatively; target glucose 140-180 mg/dL.
4. Resume home regimen when eating reliably.

*Electronically signed: Helena Varga, MD (Endocrinology) | 09/19/2026*

# Endocrinology Consultation (continued)



### Glucose Log Reviewed (patient-reported, fasting)

| Week of | Average | Range |
|---|---|---|
| 08/24/2026 | 138 mg/dL | 121-156 |
| 08/31/2026 | 134 mg/dL | 118-149 |
| 09/07/2026 | 141 mg/dL | 126-162 |
| 09/14/2026 | 136 mg/dL | 119-151 |

### Patient Education

Reviewed hypoglycemia symptoms, sick-day rules, and the plan to hold glipizide on the morning of surgery.

*Electronically signed: Helena Varga, MD (Endocrinology) | 09/19/2026*

# Patient Pre-Anesthesia Questionnaire



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Completed by patient: 09/21/2026

| Question | Answer |
|---|---|
| Prior problems with anesthesia? | Nausea after knee replacement (2019) |
| Family history of anesthesia problems? | No |
| Loose teeth, dentures, or caps? | No |
| Snoring or witnessed pauses in breathing? | Snores; spouse unsure about pauses |
| Can you climb a flight of stairs without stopping? | Yes, slowly, limited by leg pain |
| Blood thinners? | Aspirin 81 mg |

# STOP-BANG Sleep Apnea Screening



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 09/23/2026

| Item | Result |
|---|---|
| Snoring | Yes |
| Tired during the day | No |
| Observed apnea | No |
| Pressure (hypertension) | Yes |
| BMI over 35 | No |
| Age over 50 | Yes |
| Neck circumference over 40 cm | No |
| Gender male | Yes |

Score 4: intermediate risk. Continuous pulse oximetry recommended post-operatively.

# Anesthesia Pre-Operative Evaluation



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 09/23/2026 | Anesthesiologist: Marcus Lee, MD

**Planned Procedure**

Open lumbar spinal fusion, L4-L5, prone positioning, estimated 4 hours.

**Airway**

Mallampati II. Thyromental distance >6 cm. Full neck range of motion. Dentition intact.

**Cardiopulmonary**

HFrEF, EF 40%, NYHA II, compensated. Former smoker, lungs clear. STOP-BANG score 4 (intermediate risk for sleep apnea).

**ASA Classification**

ASA III.

# Anesthesia Pre-Operative Evaluation (continued)



### Anesthetic Plan

General endotracheal anesthesia. Arterial line for continuous blood pressure monitoring. Goal-directed fluid therapy given reduced EF. Multimodal analgesia with acetaminophen and regional infiltration; limit opioids.

### Post-Operative Considerations

Telemetry per cardiology. Continuous pulse oximetry given intermediate sleep apnea risk and opioid exposure.

### Clearance

Cleared for surgery with standard cardiac monitoring intra-operatively.

*Electronically signed: Marcus Lee, MD (Anesthesiology) | 09/23/2026*

# Laboratory Results



Collected: 09/23/2026

| Test | Result | Reference Range | Flag |
|---|---|---|---|
| WBC | 6.8 K/uL | 4.5-11.0 | |
| Hemoglobin | 13.2 g/dL | 13.5-17.5 | L |
| Platelets | 245 K/uL | 150-400 | |
| Sodium | 138 mEq/L | 136-145 | |
| Potassium | 4.4 mEq/L | 3.5-5.1 | |
| Creatinine | 1.1 mg/dL | 0.7-1.3 | |
| Glucose | 142 mg/dL | 70-99 | H |
| Hemoglobin A1c | 7.1 % | <5.7 | H |
| INR | 1.0 | 0.8-1.1 | |
| BNP | 210 pg/mL | <100 | H |

### Interpretation

Mild anemia, consistent with chronic disease. Hyperglycemia and elevated A1c consistent with known type 2 diabetes, adequate for surgical clearance per anesthesia. BNP mildly elevated, consistent with known compensated heart failure — no acute change from prior baseline.

# Laboratory Results — Comprehensive Metabolic Panel



Collected: 09/23/2026

| Test | Result | Reference Range | Flag |
|---|---|---|---|
| BUN | 19 mg/dL | 7-20 | |
| Chloride | 102 mEq/L | 98-107 | |
| CO2 | 26 mEq/L | 22-29 | |
| Calcium | 9.3 mg/dL | 8.6-10.3 | |
| Albumin | 3.9 g/dL | 3.5-5.0 | |
| AST | 24 U/L | 10-40 | |
| ALT | 21 U/L | 7-56 | |
| Alk Phos | 88 U/L | 44-147 | |
| Total bilirubin | 0.7 mg/dL | 0.1-1.2 | |
| eGFR | 68 mL/min/1.73m2 | >60 | |

# Laboratory Results — Coagulation, Type & Screen, Urinalysis



Collected: 09/23/2026

| Test | Result | Reference Range | Flag |
|---|---|---|---|
| PT | 11.8 sec | 11.0-13.5 | |
| PTT | 29 sec | 25-35 | |
| ABO/Rh | O positive | | |
| Antibody screen | Negative | Negative | |
| UA leukocyte esterase | Negative | Negative | |
| UA nitrite | Negative | Negative | |
| UA glucose | Trace | Negative | A |
| MRSA nares screen | Negative | Negative | |

### Interpretation

No evidence of urinary tract infection. MRSA screen negative. Type and screen valid for 72 hours.

# Laboratory Results — Prior (Primary Care)



Collected: 06/02/2026

| Test | Result | Reference Range | Flag |
|---|---|---|---|
| Hemoglobin | 13.4 g/dL | 13.5-17.5 | L |
| Creatinine | 1.0 mg/dL | 0.7-1.3 | |
| Potassium | 4.3 mEq/L | 3.5-5.1 | |
| Hemoglobin A1c | 7.3 % | <5.7 | H |
| LDL cholesterol | 78 mg/dL | <100 | |
| TSH | 2.1 mIU/L | 0.4-4.0 | |
| BNP | 195 pg/mL | <100 | H |

**Interpretation**

Stable chronic anemia and BNP compared with 2025. Diabetes control near goal.

# Laboratory Results — Complete Blood Count with Differential



Collected: 09/23/2026

| Test | Result | Reference Range | Flag |
|---|---|---|---|
| RBC | 4.41 M/uL | 4.5-5.9 | L |
| Hematocrit | 39.6 % | 41-53 | L |
| MCV | 89.8 fL | 80-100 | |
| RDW | 13.9 % | 11.5-14.5 | |
| Neutrophils | 62 % | 40-70 | |
| Lymphocytes | 27 % | 20-40 | |
| Monocytes | 8 % | 2-10 | |
| Eosinophils | 2 % | 0-6 | |

# Laboratory Trend Summary



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Prepared for pre-admission review

| Test | 01/2026 | 06/2026 | 09/2026 |
|---|---|---|---|
| Hemoglobin (g/dL) | 13.6 | 13.4 | 13.2 |
| Creatinine (mg/dL) | 1.0 | 1.0 | 1.1 |
| Hemoglobin A1c (%) | 7.4 | 7.3 | 7.1 |
| BNP (pg/mL) | 188 | 195 | 210 |
| Potassium (mEq/L) | 4.5 | 4.3 | 4.4 |

Values stable across 2026. No acute changes.

# Medication Reconciliation



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Reconciled: 09/22/2026 by Jenna Park, RN

| Medication | Dose / Frequency | Indication | Day of Surgery |
|---|---|---|---|
| Metformin | 1000 mg BID | Diabetes | Take with sip of water |
| Glipizide | 5 mg daily | Diabetes | HOLD |
| Lisinopril | 20 mg daily | HF / hypertension | Take with sip of water |
| Atorvastatin | 40 mg daily | Hyperlipidemia | Take with sip of water |
| Furosemide | 20 mg daily | Heart failure | Per anesthesia |
| Aspirin | 81 mg daily | Cardiac | Continue per cardiology |
| Gabapentin | 300 mg at bedtime | Radicular pain | Take night before |
| Acetaminophen | 1 g TID PRN | Pain | May take |

Patient reports no herbal supplements. Last dose times to be confirmed at check-in.

# Pre-Operative Orders



| | |
|---|---|
| **Procedure:** | Open lumbar spinal fusion, L4-L5 |
| **Scheduled Date:** | 10/02/2026 |
| **Admission Type:** | Inpatient |
| **Attending Surgeon:** | Amanda Reyes, MD |

### Pre-Operative Instructions

1. NPO after midnight prior to surgery.
2. Continue metformin, lisinopril, atorvastatin morning of surgery with sip of water.
3. Hold glipizide morning of surgery.
4. Type and screen, CBC, BMP, coagulation panel day of surgery.
5. Cardiology clearance on file — see attached.
6. Anesthesia pre-op evaluation on file — see H&P.
7. Sequential compression devices intra-operatively for DVT prophylaxis.

# Pre-Operative Orders (continued)



**Post-Operative Plan (Anticipated)**

Patient anticipated to require inpatient admission post-operatively for pain control, mobilization, and cardiac monitoring given history of heart failure. Physical therapy consult day of surgery. Telemetry monitoring per cardiology recommendation. Diabetic management per sliding scale insulin protocol while NPO/post-operative. Discharge planning to begin day 1 post-operatively; anticipated discharge disposition to be determined based on mobility and pain control status.

**DVT Prophylaxis**

Sequential compression devices + subcutaneous heparin per protocol, starting post-operative day 0.

## Post-Operative Order Set — Pain Management



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Pre-signed by Amanda Reyes, MD, 09/24/2026

1. Acetaminophen 1 g IV/PO every 8 hours scheduled.
2. Gabapentin 300 mg at bedtime, resume home dose.
3. Hydromorphone PCA per protocol on post-operative day 0, transition to oral oxycodone as tolerated.
4. Avoid NSAIDs given heart failure and fusion.
5. Continuous pulse oximetry while on PCA.
6. Bowel regimen: senna and docusate.

## Therapy & Equipment Orders



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Pre-signed by Amanda Reyes, MD, 09/24/2026

1. Physical therapy evaluation day of surgery; ambulate with assistance as tolerated.
2. Occupational therapy evaluation post-operative day 1 for ADLs and bathing.
3. Spine precautions: no bending, lifting over 10 lb, or twisting.
4. Durable medical equipment at discharge: front-wheeled walker, raised toilet seat, reacher.
5. Stair training before discharge.

## Informed Consent for Surgery



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213

I consent to the following procedure: open lumbar spinal fusion, L4-L5, with posterior instrumentation and decompression, to be performed by Amanda Reyes, MD, and assistants.

The nature of the procedure, its expected benefits, material risks (including infection, bleeding, nerve injury, spinal fluid leak, failure of fusion, blood clots, heart complications, and death), and alternatives have been explained to me. I have had the opportunity to ask questions.

I consent to the administration of blood products if necessary.

*Patient signature: John Doe | Witness: Jenna Park, RN | 09/22/2026*

## Consent for Anesthesia



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213

I consent to general anesthesia and invasive monitoring as described by my anesthesiologist. Risks including sore throat, dental injury, nausea, awareness, allergic reaction, heart and lung complications have been explained.

*Patient signature: John Doe | Anesthesiologist: Marcus Lee, MD | 09/23/2026*

# Patient Education — Preparing for Spine Surgery



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Education completed: 09/22/2026

**Topics Reviewed**

• Nothing to eat or drink after midnight the night before surgery.
• Which morning medications to take with a sip of water, and to hold glipizide.
• Shower with chlorhexidine soap the night before and morning of surgery.
• Arrange a responsible adult to drive you home and stay with you after discharge.
• Prepare your home: clear walkways, place frequently used items at waist height, and plan for help with stairs.
• Bring your cane, medication list, and insurance card on the day of surgery.

**Teach-Back**

Patient and spouse verbalized understanding of all instructions.

*Jenna Park, RN* | *09/22/2026*

# Nursing Pre-Admission Assessment



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213 | Date: 09/22/2026

| | |
|---|---|
| **Height / Weight:** | 5 ft 11 in / 208 lb (BMI 29.0) |
| **Fall Risk:** | Moderate — uses cane, radicular weakness |
| **Skin:** | Intact |
| **Pain:** | 6/10, right leg |
| **Support at Home:** | Spouse; adult daughter nearby |
| **Advance Directive:** | Yes — copy on file |

### Discharge Planning Screen

Lives in a two-story home with bedroom upstairs. Spouse able to assist. Anticipate walker and raised toilet seat at discharge. Physical therapy to assess for home versus short-term rehabilitation.

*Jenna Park, RN* | *09/22/2026*

# Consent for Blood Transfusion



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213

I consent to transfusion of blood or blood products if my doctor finds it necessary during or after surgery. Risks, benefits, and alternatives, including cell salvage, were explained.

*Patient signature: John Doe | Witness: Jenna Park, RN | 09/22/2026*

# Authorization to Release Health Information



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213

I authorize Ridgeline Orthopedic & Spine Associates to release my medical records to Concord Health Plans for the purpose of prior authorization and payment for my planned surgery. This authorization expires one year from the date signed.

*Patient signature: John Doe | 09/18/2026*

# Financial Responsibility Acknowledgment



Patient: John Doe | DOB: 03/14/1955 | MRN: 774213

I understand that my surgery requires prior authorization from my health plan. I understand I am responsible for my plan's copays, deductible, and coinsurance, and that the facility will provide an estimate once a decision is made. I will be contacted if the authorization is not approved.

*Patient signature: John Doe | 09/22/2026*

# Member Eligibility & Benefits Verification (RESENT)



| | |
|---|---|
| **Member Name:** | John Doe |
| **Member ID:** | MBR-0042 |
| **Group Number:** | GRP-88214 |
| **Effective Date:** | 01/01/2026 |
| **Plan Type:** | Concord Gold Plus HMO (Medicare Advantage) |
| **PCP of Record:** | Michael Tran, MD — Lakeview Family Medicine |
| **Prior Auth Required:** | Yes — inpatient surgical admission |
| **Inpatient Benefit:** | $350 copay per admission |
| **Annual Deductible:** | $500 (in-network); $180 met YTD |
| **Out-of-Pocket Max:** | $4,900 (in-network); $1,265 met YTD |
| **Verification Date:** | 09/20/2026 |
| **Verified By:** | K. Alvarez, Patient Access Rep |

Note: Eligibility and benefit information is verified as of the date shown above and is subject to change. Verification does not guarantee payment.

# Provider Attestation & Certification of Medical Necessity



I certify that the information provided in this prior authorization request, and the attached clinical documentation, is accurate and reflects the current medical condition of the patient named herein. I attest that the requested inpatient admission for open lumbar spinal fusion (L4-L5) is medically necessary based on the patient's clinical presentation, failed conservative management, and comorbid conditions requiring inpatient-level post-operative monitoring.

**Provider Signature:** Amanda Reyes, MD

**Date:** 09/24/2026

**NPI:** 1780234561

— End of transmission (106 pages) —